

escalated, she felt out of control again. When she used impulsively after a tough day, the critic went on the warpath in a thankless effort to shame her firefighter part into reforming. Unfortunately, this amplified her exiles' sense of worthlessness and motivated her firefighter to lean more heavily on the calming effects of cannabis. Since the critic's desperate efforts to control the firefighter were making matters worse, the first order of business in this session was to help it feel less alone and more connected to Gina's Self.

As Gina illustrates, we can facilitate dialogues with insistent critics. First, we ask them how they're trying to help. Critics in addictive systems typically want to prevent more of the chaos and disasters that firefighters engineer, which is surely positive. As the manager team unblends and develops an ongoing, trusting connection with the Self, we can guide the client to ask some pointed questions, ever so lightly and with affection. As hard as these managers work, have they managed to fix or even discourage firefighters? The client often gets a rueful acknowledgment that their tactics haven't worked, so plan B might be a good idea. As this path opens, the client turns their attention back toward firefighters with the curiosity and calm of Self-energy.

### QUESTIONS THE THERAPIST CAN ASK CLIENTS ABOUT THEIR CRITICAL MANAGERS

- Where does this critic show up in the body?
- What kind of energy does it have? How does it feel?
- Do you see it? If so, what does it look like?
- Do you hear it? If so, do you hear it inside or outside, and what does it sound like?

### QUESTIONS CLIENTS CAN ASK THEIR CRITICAL MANAGERS

- Which part(s) are you attacking?
- Which parts worry you most?
- When you go after \_\_\_\_\_ part, how are you trying to help?
- What would happen if you stopped going after that part? Have you seen that happen before?
- What if firefighters are more complex than they seem? If you saw there was another way to help this firefighter so it used its energy in a better way, would that give you a break?
- If we could retrieve exiles and they could let go of their burdens, would you still have to work so hard?

- What do you want to say to me?
- How old are you?
- How old do you think I am?
- What would it be like if you could do this with me and didn't have to handle it alone?

## REORIENTING BACK TO FIREFIGHTERS

As the client refocuses their attention back to firefighter parts, they might find that some are so walled off they seem to be on autopilot. Clients may believe that the urges and compulsivity of these parts hinder them from trying new options. In reality, though, these parts are not incapable of negotiating, nor are they permanently unreachable. They just haven't been reached yet. We can engage them with direct access.

For example, Ida was a forty-four-year-old, Polish American, single, cisgender woman who first came to therapy to curb her drinking. Her father had been chronically ill since she was a child, and Ida had been his main caretaker. She reported a pattern of working feverishly for 10 to 12 days at a time and then spending a week or so holed up in her house, drinking nonstop. When Ida was working, she usually showed up to therapy, but whenever she was drinking, she always canceled. She was worried about her health but felt unable to change her drinking habits.

The therapist invited her to identify the central polarity between her manager and firefighter teams. Her driven, compulsive manager team included a workaholic part, some driven caretaker parts, and a team of harshly judgmental critics who shamed her for drinking. Her manager crew was balanced by a compulsive firefighter team, which included a very committed drinking part, a rebellious angry part who felt like life was unfair, and a group of defensive parts who studiously avoided hearing feedback about her compulsive drinking and work habits. She had a few memories and some awareness of having been lonely when she was a child, but she had no genuine access to her early experiences and described herself as emotionally shut down.

### REORIENTING TO A FIREFIGHTER



**Therapist:** We've been focused on the big polarities in your system, and we've seen how inner battles keep you feeling stuck. You've talked with some harsh, shaming managers who have given you some space, which is great! At the same time, we're not so connected with the part who is committed to drinking. We could just invite that part to be with us right now. She's always welcome in our sessions. If she came here with you, we could get to know her.

**Ida:** You mean I should show up here drunk?

**Therapist:** Well, I'd still like to be able to talk with you and her. But yes, I'm serious about inviting her to visit with us, drunk or sober. She has been shamed and hidden for so long. I want her to feel welcome.

**Ida:** (*She laughs and shrugs.*) Well, okay, maybe I'll do just that!

**Therapist:** No driving drunk, though, right? An Uber or a taxi. (*Ida nods.*)



Two weeks later, Ida showed up at the session and said she'd been drinking. The therapist welcomed the drinking part and lauded her courage for showing up despite a long history of having been judged. Ida said her drinking part wanted to talk with the therapist directly, which is what we call explicit direct access.



**Therapist:** So, you're the part of Ida who drinks a lot. I'm happy to have this opportunity to talk with you. I'd love to hear all about you. For example, how old were you when you got interested in drinking?

**Ida's Drinking Part:** Thirteen. Me and my girlfriends sat up one night drinking a handle of vodka from the liquor cabinet in this one girl's house. We put it in orange juice and 7UP—and it tasted great! It was the first time I ever had fun. We laughed and ran around the house and acted crazy. It was a blast. I was sicker than a dog the next day, which was awful, but I knew I'd do it again as soon as I could.

**Therapist:** Uh-huh. It sounds like you were able to forget everything and just be wild kids, is that right? (*Ida nods.*) And it was unusual for you to have fun with friends.

**Ida's Drinking Part:** Oh yeah, my dad was so sick. He was bedridden my whole childhood with one thing or another. Since my mother hated taking care of him, it was up to us. We came right home from school and got him to the bathroom. I knew he was waiting. It was kinda terrible.

**Therapist:** Mm-hmm. So much pressure on Ida to be responsible. So you were the part who helped her get a little down time, be a little lighthearted, and act like a kid.

**Ida's Drinking Part:** I guess I was! Otherwise, it was just depressing. So hopeless. Nothing to look forward to—like ever.

**Therapist:** Right, so do you think there were some even younger or more tender parts who felt hopeless about anything ever getting better?

**Ida's Exile:** I'm still hopeless.

Ida is now blended with the hurting, hopeless exile, along with the drinking part.

**Therapist:** Right, yes of course. You've felt hopeless for a long time. It's like it never goes away. We're going to help you. But I wonder if you could be patient for just a little longer. We need permission from the drinking part first. Is that okay?

**Ida's Exile:** Okay.

**Therapist:** Great. Thank you. So now I want to talk with the drinking part again. (*Ida nods.*) I wonder, as the part of Ida who drinks, if you're aware that drinking keeps that hopelessness quiet?

**Ida's Drinking Part:** Well, I know if she didn't drink, she'd be miserable.

**Therapist:** Right. You've been protecting Ida from feeling hopeless and miserable. I know it would be tough and scary to stop. Your help has been essential! But now if Ida could help that hopeless part so it felt stronger and not so alone, would you get a break?

**Ida's Drinking Part:** I guess so. But I'm not sure anyone can help.

**Therapist:** Right, you'd need evidence of that. Ida and I would need to show you. And, with your permission, we will.



This session informed Ida about the benign intentions of her drinking part and opened her heart to its feelings. It also introduced her to a key exile. While it is unconventional to invite a client to come to—or stay in—a session while engaging in their compulsive behavior, it can serve to reassure parts quickly and powerfully that we mean what we say: All parts are welcome. We won't judge.

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